SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

carboplatin AUC 6 / paclitaxel → dose-dense AC

Triple-negative breast cancer · neoadjuvant chemotherapy · NCCP 00348 · version 8a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Treat the cancer before surgery as part of a plan intended to reduce the risk of it returning.

Schedule
Phase 1: four 21-day cycles; paclitaxel on Days 1, 8 and 15, carboplatin on Day 1. Phase 2: four AC cycles every 14 days. Surgery and pathology determine the next step. See the whole-treatment map for monitoring and sequencing.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Surgery and postoperative plan discussed
- ☐ Blood tests, ECG / cardiac and imaging checks explained
- ☐ Common, serious and lasting medicine-specific risks discussed below
- ☐ Platinum/taxane and AC phases explained
- ☐ Separate supportive prescriptions and growth-factor plan agreed
- ☐ Fertility, contraception and pregnancy precautions discussed
- ☐ Questions and the individual consent discussion recorded

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Allergic reaction, sometimes after previous exposure
- ☐ Kidney or electrolyte changes

Paclitaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Numbness / tingling, sometimes lasting
- ☐ Hair loss, muscle or joint pain
- ☐ Infusion reaction
- ☐ Mouth, bowel, skin and nail effects

Doxorubicin
ANTHRACYCLINE CHEMOTHERAPY

- ☐ Low counts, infection and mouth ulcers
- ☐ Nausea, hair loss and fatigue
- ☐ Heart damage, sometimes delayed
- ☐ Severe tissue injury if the infusion leaks
- ☐ Rare later blood cancers

Cyclophosphamide
ALKYLATING CHEMOTHERAPY

- ☐ Low counts and infection
- ☐ Nausea, vomiting and hair loss
- ☐ Bladder irritation / bleeding
- ☐ Fertility effects and menopause
- ☐ Rare later blood cancers

Discuss treatment aim, alternatives, expected benefit, common and serious risks, fertility, pregnancy precautions and the full sequence. The treating team completes the individual consent discussion; this guide does not determine personal benefit.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00348 v8a.

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